

THIS WEEK AT A GLANCE

The Ebola Bundibugyo PHEIC enters a stabilisation-and-countermeasures phase: 282 confirmed cases and 42 confirmed deaths, with candidate therapeutics now prioritised for emergency trials. US measles has reached 1,983 cases — elimination status formally at risk. H5N1 avian influenza records a third unknown-exposure case among US humans, heightening pandemic-preparedness concern.

Bottom line: The **Bundibugyo ebolavirus** outbreak in DRC and Uganda enters a critical transition from emergency response to countermeasure deployment. As of 31 May, DRC reports **282 confirmed cases** across 17 health zones in Ituri, North Kivu and South Kivu, plus 220 suspected cases under investigation, with **42 confirmed deaths**. Five people have now recovered — including four nurses — a first positive signal. WHO and Africa CDC have prioritised three candidate therapeutics for emergency-use trials: the monoclonal antibodies **MBP 134** and **maftivimab**, and the antiviral **remdesivir**. The oral antiviral **obeldesivir** is being prioritised for post-exposure prophylaxis, and two candidate BVD vaccines have been identified for clinical evaluation. Cross-border risk remains elevated: Uganda has 9 confirmed cases and 1 death, all linked to DRC travel. US measles surpassed **1,983 confirmed cases** as of 28 May — across 40 US jurisdictions, 30 active outbreaks — putting elimination status formally at risk for the first time since 2000. H5N1 avian influenza has now recorded **3 US human cases** with unknown animal-exposure source out of 71 total, raising concern about gaps in surveillance and possible cryptic spread.

ACUTE EVENT · LEAD STORY · PHEIC			LEAD		
Ebola Bundibugyo					
Bundibugyo ebolavirus (BVD) — DRC & Uganda PHEIC · countermeasure trials underway					
DRC: Ituri (264 confirmed), North Kivu (15), South Kivu (3) · Uganda: 9 confirmed, 1 death					
282		42		5	
CONFIRMED CASES (DRC + UGANDA, 31 MAY)		CONFIRMED DEATHS		RECOVERIES — FIRST POSITIVE SIGNAL	
Severity 9.5					
Novelty 8.0					
Attention 9.0					
Policy 10.0					
Pandemic 4.5					
Composite: 8.2 / 10					
WHY IT MATTERS					
The outbreak has shifted from explosive growth into a high-intensity stabilisation phase. As of 31 May 2026, DRC reports 282 confirmed cases in 17 health zones across Ituri (264), North Kivu (15) and South Kivu (3), plus 220 suspected cases under investigation. Uganda has 9 confirmed cases and 1 death. Total confirmed deaths stand at 42. Critically, 5 people have recovered — including 4 nurses — a first clear evidence that survival with supportive care is achievable. WHO and Africa CDC R&D Blueprint have shortlisted candidate countermeasures for emergency-use clinical trials: monoclonal antibodies MBP 134 and maffivimab, remdesivir (IV/oral), obeldesivir (oral; targeted for post-exposure prophylaxis), and two BVD vaccine candidates. The Title 42 entry restriction (DRC, Uganda, South Sudan) remains in force. Nosocomial transmission in health-care settings continues to be a key driver; IPC training and PPE supply remain critical gaps.					
FOR POLICYMAKERS					
• Accelerate WHO/Africa CDC R&D Blueprint emergency-use clinical-trial protocols for MBP 134, maffivimab and remdesivir in DRC and Uganda; coordinate regulatory fast-track pathways so enrolment can begin within days, not weeks.					
• Operationalise obeldesivir post-exposure prophylaxis for high-risk healthcare contacts; data from the five confirmed recoveries should inform supportive-care protocols for treatment centres in Bunia, Mongbwalu and Kampala.					
• Maintain cross-border exit screening between DRC, Uganda, Rwanda, South Sudan, CAR and Kenya; sustain Title 42 screening at US ports of entry while avoiding broad travel-restriction language that depresses voluntary healthcare-seeking.					

SMOLDERING · ELIMINATION STATUS AT RISK		
Measles (USA)		
Measles morbillivirus — largest US outbreak since 2019; elimination status formally at risk		
40 US jurisdictions · 30 active outbreaks · Utah, Texas, Ohio, California leading		
1,983	30	93%
CONFIRMED US CASES IN 2026 (AS OF 28 MAY)	ACTIVE OUTBREAKS IN 2026	CASES THAT ARE OUTBREAK-ASSOCIATED
Severity 4.5		
Novelty 3.0		
Attention 7.0		
Policy 9.0		
Pandemic 3.5		
Composite: 5.4 / 10		
WHY IT MATTERS		
With 1,983 confirmed cases across 40 US jurisdictions as of 28 May 2026, the US is tracking closely to its full-year 2025 total of 2,288 — with seven months of 2026 remaining. Thirty new outbreaks have been identified in 2026 alone (93% of cases are outbreak-associated), and epidemiologists have publicly flagged that elimination status — held since 2000 — is formally at risk. Nine of the 1,983 cases are international visitors, confirming ongoing importation risk. The core driver is declining MMR vaccination coverage in select communities and states. A spike in post-outbreak vaccination rates has been documented, but structural coverage gaps in unvaccinated communities remain.		
FOR POLICYMAKERS		
• Immediately assess MMR vaccination coverage at county level in all 40 affected jurisdictions and identify sub-80% coverage pockets — those are the outbreak accelerants that must be addressed with targeted catch-up campaigns.		
• Activate state emergency vaccination authority where outbreaks are active: school-exemption review, mobile vaccination teams, and community partnership with faith and school networks in hesitant communities.		
• Coordinate with CDC, APHA and ASTHO to formally communicate the elimination-status risk threshold — policymakers need to understand that the US could lose WHO measles-elimination certification if current trends continue.		

WATCH · PANDEMIC POTENTIAL		
H5N1 Avian Influenza		
Influenza A(H5N1) — 71 US human cases; 3 unknown-exposure; novel H5N5 human case		
USA (dairy cows, poultry workers) · Bangladesh (fatal paediatric case) · global wild-bird reservoir		
71	3	2
US HUMAN CASES SINCE 2024 (AS OF 27 MAY)	CASES WITH UNKNOWN ANIMAL-EXPOSURE SOURCE	US DEATHS
Severity 7.0		
Novelty 5.5		
Attention 5.5		
Policy 6.5		
Pandemic 9.0		
Composite: 6.7 / 10		
WHY IT MATTERS		
The US multistate H5N1 outbreak in dairy cows and poultry continues to generate sporadic human cases. As of 27 May 2026, 71 US human infections have been confirmed since 2024, with 2 deaths. The most concerning signal this week: 3 of those 71 cases have no identified animal-exposure source — raising the possibility of missed exposures, environmental exposure, or limited undetected human-to-human transmission. No sustained H2H spread has been documented. Globally, WHO tracked 13 avian and swine influenza cases in humans in Q1 2026, including a fatal H5N1 case in a child in Bangladesh. A NEJM report documents the first confirmed human infection with H5N5 — a novel variant requiring close monitoring. Scientists have publicly warned that H5N1 could spark a human pandemic in 2026.		
FOR POLICYMAKERS		
• Urgently investigate all 3 unknown-exposure US H5N1 cases with full environmental and contact-tracing workups; any evidence of human-to-human transmission must trigger immediate escalation to pandemic preparedness activation.		
• Strengthen surveillance in dairy-farm workers: serological screening beyond symptomatic testing — asymptomatic H5N1 cases are likely being missed given the unknown-exposure detections.		
• Accelerate H5N1 pandemic vaccine candidate production and pre-positioning; WHO and HHS should review current CVV stock against circulating H5 clade 2.3.4.4b strains and update if antigenic drift is detected.		